



Nursing Practice Guideline

BOWEL PROCEDURES

Reference #: NPG 63

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to bowel procedures. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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Principles

Observe the five moments for hand hygiene. Follow aseptic technique and use the risk assessment outlined in the outlined in SCGOPHCG Infection Prevention and Management Manual - Aseptic Technique. This must include use of appropriate Personal Protective Equipment (PPE) to prevent contamination and mucosal or conjunctival splash injuries.

Patient identification must be checked when matching patient's identity to care, therapy and services.

It is a legal requirement that all disciplines that prescribe or administer medications comply with the Medicines and Poisons Act 2014, Medicines and Poisons Regulations 2016, Pharmacy Act 2010, and Hospital Guidelines.

Only valid prescriptions will be utilised by any nursing staff to administer medication except for verbal orders and nurse-initiated medications, as per designated list.

1. Suppositories, Enemas & Digital Removal of Faeces

1.1 Medical Requirements

A rectal examination may need to be carried out if deemed necessary by the medical officer prior to the administration of these treatments. The procedure for digital removal of faeces is to be requested by the medical officer after medical assessment (including rectal exam), in consultation with nursing staff and should be supported by documentation in the in-patient notes.

Discuss an appropriate analgesic plan with medical officer prior to performing digital removal of faeces.

All enemas / suppositories must be charted on the WA Hospital Medication Chart.

1.2 Safety Information

Life threatening dehydration and/or electrolyte disturbances may occur in "at risk" groups (e.g. patients with diarrhoea, cardiac arrhythmias, recent myocardial infarction) undergoing regular or frequent bowel treatments. (See EMIMS under Contraindications)

Bowel dysfunction whether faecal incontinence or constipation is often embarrassing and demoralising for a patient. Good communication is central to ensure the correct differential diagnosis is reached and patients and carers are involved in planning care.³

Staff should be aware of any cultural, religious beliefs or restrictions which may prohibit any bowel procedures being done by a member of the opposite sex. Patients should be asked if they request a chaperone when undergoing any intimate procedure or examination.¹

Treatment for constipation may increase faecal urgency and in older adults who require toileting assistance this can increase their risk of falls. Consider strategies to reduce risk from falls when treating constipation.

Digital rectal examinations and digital removal of faeces should not be performed in patients with history of the following⁹. In these cases, please refer to Medical Officer:

- Neutropenia
- Thrombocytopenia
- Recent history of colorectal surgery
- Recent prostate surgery

Special consideration should be given to patients with the following medical issues. Refer to local medical staff for further instructions in these cases:

- Paralytic Ileus,

- Colonic obstruction,
- Perforation of the bowel,
- Rectal pain,
- Rectal bleeding,
- Anti-clotting medication,
- Spinal injury at level of thoracic T6 as autonomic dysflexia can occur. Observe for signs and symptoms of hypertension, tachycardia, sweating and hot flushes.

Digital rectal examination is an important assessment that can aid selection of appropriate treatments e.g. if the rectum is empty rectal preparations may not be appropriate.

1.3 Nursing Management

- Allow sufficient time for effective analgesic cover prior to commencement of digital removal of faeces. An inhalation analgesic or local anaesthetic gel may be appropriate.
- Conduct a physical assessment of the abdomen and rectum. Assess for bowel sounds, haemorrhoids, faecal impaction and intact anal reflex. "The anal reflex occurs when touching skin by the anus; it contracts then relaxes".¹ The perianal area should also be examined for signs of impaired skin integrity, infestations, bleeding, foreign bodies, wounds or discharge. Any abnormalities should be reported to the medical officer prior to performing any bowel interventions¹¹.
- If inserting suppositories or enemas and the nurse feels a faecal mass, ensure the mass is bypassed. If the patient cannot pass the faecal matter post suppositories or enema, then digital removal may be required.
- Post-procedure: Perform a bladder scan to monitor for high residual volume of urine. If the total bladder volume is > 800mL, notify Medical Officer, as per NPG Bladder Management and Urinary Catheterisation.
- Document result in patient's notes and provide appropriate clinical handover of bowel interventions and ongoing bowel management plan⁹.
- Provide advice and education regarding ongoing dietary and bowel management strategies/programmes throughout hospital stay and upon discharge to patient and/or care giver.

1.4 Procedural Information

1.4.1 Suppository Administration

Requirements

- Personal Protective Equipment (PPE)
- Lubricant
- Suppository
- Tissues
- Disposal Bag
- Absorbent sheet

Procedure

1. Perform a 3-point identification check of the patient and check the type of suppository to be administered.
2. Explain the procedure and obtain patient consent.^{1,4}
3. Ensure privacy of the patient.
4. Ensure the bedpan, commode or toilet is easily accessible if required.²

5. Perform hand hygiene.
6. Don PPE.
7. Place absorbent sheet under patient and position in left lateral position with knees flexed and the right knee slightly higher than the left ¹⁰.
8. Remove suppository from wrapper.
9. Place some lubricating jelly on the apex end of the suppository if used for local effect (e.g. bowel evacuation) or the blunt end if the suppository is used for systemic effect (e.g. analgesic)^{1,8}. *Nb: Glycerine suppositories should be moistened with water not lubricant according to manufacturer's instructions.*
10. Separate the patient's buttocks and gently insert suppository into the rectum. Advance for approximately 2-4 centimetres into the rectum.⁴
11. Repeat this procedure if a second suppository is required.
12. Clean any excess lubricating gel from perianal area. If the patient has a desire to expel the suppository, hold the buttocks firmly together until the spasm has passed.
13. Encourage patient to retain suppository for at least 20 minutes if possible to allow for full absorption. If the suppository has been administered for other purposes, the patient is unlikely to feel the urge to dispel it.⁴
14. If suppository was intended for evacuation of bowels, give assistance to the patient with toileting as required.
15. Dispose of waste in appropriate bin.
16. Remove gloves and perform hand hygiene.
17. Sign WA Hospital Medication Chart and document in patient notes that a suppository has been given, the results and stool type as per Bristol Stool chart.

1.4.2 Enema (Small Volume Enema) Administration

Requirements

- Personal Protective Equipment (PPE)
- Lubricant
- Enema
- Tissues
- Disposal Bag
- Absorbent sheet

Procedure

1. Perform a 3-point identification check of the patient and check the type of enema to be administered.
2. Explain the procedure and obtain patient consent.^{1,5}
3. Ensure privacy of the patient.
4. Ensure the bedpan, commode or toilet is easily accessible ².
5. Allow patient to empty bladder first if necessary.
6. Perform hand hygiene.
7. Don PPE.
8. Place absorbent sheet under patient and position in left lateral position with knees flexed and the right knee slightly higher than the left ¹⁰.
9. Remove cap of enema or external wrapping and lubricate the entire length of the enema tip. **Nb:** Fleet enema are pre-lubricated, further lubrication is not required.

10. Ensure the opening of the tip remains free from obstruction so that the solution flows freely when administering.⁵
11. As the sphincter relaxes, gently insert enema as per instructions into the rectum and allow the solution to flow whilst observing for any signs of pain/discomfort. If the patient cannot relax or there is any resistance for any other reason, do not force the enema into the rectum.⁵
12. Ask patient to remain in the current position and retain the enema for as long as possible until they have a strong urge for a bowel movement.⁵ Give assistance to the patient, with toileting as required.
13. Dispose of waste.
14. Remove gloves and perform hand hygiene.
15. Sign WA Hospital Medication Chart and document that an enema was given and the results. Report any abnormalities to senior staff.

1.4.3 Digital Removal of Faeces

Digital removal of faeces may be undertaken in the following circumstances:

- When other methods of bowel emptying fail or are inappropriate
- Faecal impaction or loading
- Incomplete defecation
- Inability to defecate
- Neurogenic bowel dysfunction
- In many patients with spinal cord injury.^{1,3}

Requirements

- Personal Protective Equipment (PPE)
- Lubricant
- Tissues
- Disposal Bag
- Absorbent sheet

Procedure

1. Perform a 3 point identification check of the patient.
2. Explain the procedure and obtain patient consent.^{1,3}
3. Ensure privacy of the patient. Perform hand hygiene.
4. Don PPE.
5. Place absorbent sheet under patient and position in left lateral position with knees flexed and the right knee slightly higher than the left ¹⁰.
6. Gently insert gloved and lubricated index finger beyond the anal sphincter until obstruction is encountered.
7. Gently remove all faecal material with which finger comes into contact.
8. After evacuating faeces wash and dry rectal area.
9. Dispose of waste appropriately.
10. Remove gloves and perform hand hygiene.
11. Document in inpatient notes that digital removal of faeces was carried out, the result and stool type as per Bristol Stool chart.
12. Ensure medical officer has prescribed ongoing oral aperients and repeat suppositories/enemas to clear the colon effectively. (May need to be repeated for a several days).

2. Flatus/Rectal Tube Management

A flatus/rectal tube may be inserted into a patient's rectum to facilitate the expulsion of flatus and the excretion of stools for individuals with impaired bowel motility. This may be the result of abdominal surgery, bowel investigations, spinal cord injury, or age.⁶

2.1 Medical Requirements

The procedure is to be requested by the Medical Officer following medical assessment of the bowel and clearly documented in patient notes. Verbal consent from the patient must be obtained prior to performing this procedure.

2.2 Safety Information

If acute pain occurs, stop procedure and inform medical staff immediately.

If Foley catheter used, DO NOT inflate balloon.

2.3 Nursing Management

As per procedural instructions

2.4 Procedural Information

Requirements

- Flatus Tube or Urethral Foley catheter size 20 FG/24FG
- Lubricant
- Bowl of water at room temperature
- Absorbent sheet
- Tissues
- Personal Protective Equipment (PPE)

Procedure

1. Perform a 3-point identification check of the patient.
2. Explain the procedure and obtain patient consent.⁶
3. Ensure privacy of the patient.
4. Perform hand hygiene.
5. Don PPE.
6. Place absorbent sheet under patient and position in left lateral with buttocks near the edge of the bed or in knee to chest position.⁶
7. Gently insert gloved and lubricated finger and assess amount of faecal loading. If the rectum is full, do not proceed, refer to medical team. If the rectum is empty, continue with the procedure.
8. Reassure the patient and instruct them to breathe through the mouth during insertion of lubricated catheter.
9. Gently insert the lubricated catheter approximately 10cms- into the rectum. (do not inflate the balloon if using a Foley catheter).⁶
10. Closely observe the patient and ask them to report any pain. If pain occurs, cease procedure immediately and report to medical staff.
11. Place distal end of tube into the bowl of water and watch for air bubbles to assess for successful release of gas.
12. Allow tube to remain in place until bubbling stops or as ordered by medical officer.⁶

13. Remove tube, clean surrounding skin and provide perianal care as required, leaving patient dry and comfortable.
14. Dispose of waste into the appropriate bin.
15. Remove gown, gloves and perform hand hygiene.
16. Document the procedure and any results in the inpatient notes and ensure adequate nursing handover.

3. Flexi-Seal™ Faecal Management System (SCGH only)

The Convatec Flexi-Seal™ faecal management system is a specialised product that aims to effectively divert and contain liquid and semi-liquid stool away from the body.

The system's goals are to reduce the risk of skin breakdown by helping to keep the skin clean, dry and free from contaminants, reduce the spread of infection by containing infectious body waste within a closed system, protect wounds, surgical sites and burns from contamination by stool, help improve patient comfort, help reduce the cost of managing faecal incontinence and help maintain patient dignity.⁷

3.1 Medical Requirements

A written order for insertion should be documented, after reviewing indications/contraindications for Flexi-Seal™.

Aperients and gut motility drugs should be considered and be prescribed as appropriate / required to ensure stool remains soft / liquid whilst Faecal Management system remains insitu.

3.2 Safety Information

Notify a medical staff immediately if any rectal pain, bleeding or abdominal distension and is present.

If acute pain occurs, during insertion, irrigation or when collecting stool samples immediately stop the procedure and inform medical staff.

Flexi-Seal™ is not intended for use in paediatric patients.

Flexi-Seal™ is not intended for use for more than 29 consecutive days⁷

Contraindications:

- refer to the Medical Officer if the patient has had one or more of the following:
- Surgery to the large bowel or rectum within the last year.
- Sensitivity or a previous allergic reaction to any components within the kit.⁷
- Rectal or anal injury.
- Rectal or anal stricture or stenosis
- Rectal or anal stricture, or stenosis
- Suspected or confirmed rectal mucosa impairment (e.g. severe proctitis, ischaemic proctitis, and mucosal ulcerations).
- Suspected or confirmed rectal or anal tumour.
- Haemorrhoids of significant size.⁷
- Faecal impaction (see page 2 for further information).
- Any in-dwelling rectal or anal device (e.g.; rectal thermometer,) or need for rectal procedures (e.g.; suppositories, enemas).⁷

3.3 Nursing Management

3.3.1 Insertion

- The Flexi-Seal™ system is only appropriate for non-ambulatory patients, who have liquid to semi liquid stool, persistent for >12hours, despite medical review and efforts to rectify cause of diarrhoea.
- Patients should have the cause of their liquid stool assessed to see if the cause can be rectified (review medications, send sample for stool culture, review enteral feeding and antibiotic regimes). Faecal impaction can cause overflow of liquid stool. Prior to insertion check for impaction by performing a rectal exam.
- Flexi-Seal™ insertion procedure requires two staff members (an operator and assistant), one of which must be a Registered Nurse.
- The two staff is required for the procedure until the balloon is inflated and catheter position is confirmed.

3.3.2 Care and Maintenance

- The clinical need for the Flexi-Seal™ catheter must be reviewed daily and documented in the patient notes (i.e. Leave insitu if draining faecal fluid. Consider removal if catheter is only draining irrigation solution).
- Observe the catheter two hourly for kinks or faecal obstruction. If the patient is placed in a sitting position, drainage may be obstructed and leakage is likely around the catheter.
- Inspect the catheter near the anus 4hourly and observe for any changes to the position of the black indicator line (which may indicate twisting of the device).
- Ensure no traction is placed on the catheter when patient is re-positioned which may cause trauma.
- Milk the catheter as required maintaining unobstructed flow to the catheter bag.
- Change the collection bag as needed.
- Change the catheter after 29 days insitu. Note that treatment with a Flexi-Seal™ is contraindicated for periods greater than 29 days, seek advice from medical officer if continuation of treatment required.
- If the collection bag fills with air, disconnect the bag and expel air as needed, then reconnect.
- Retention cuff does not require routine deflation / checking, however if a leak occurs check position then deflate and reinflate cuff.
- Based on clinical indication aperients must be considered and prescribed regularly or prn to ensure that stool remains soft / liquid.

3.3.3 Irrigation

- The purpose of the irrigation is to perform controlled bowel evacuations and minimise leakage between irrigations.
- Routine irrigation is required at least every 4 hours. Frequency can be increased if the consistency of the faecal material is becoming thicker.

The flexi-seal™ catheter is irrigated a minimum of 4 hourly using 50-100ml lukewarm tap water. Ensure irrigation is delivered through blue port marked 'IRRIG/Rx'.

3.3.4 Removal

- The catheter is non-sterile and may be reinserted if accidental removal of catheter

occurs or if device is expelled within 29 days. Staff are to use their discretion when considering re-insertion of accidentally removed catheter or use of a new catheter. Dispose of Flexi-Seal™I catheter appropriately after use.

- Review medication orders for aperients when planning catheter removal.

3.4 Procedural Information

3.4.1 Insertion of Flexi-Seal™

Requirements

- Flexi-Seal™ Management System
- Personal Protective Equipment (PPE)
- Water soluble lubricant / topical anaesthetic gel
- Absorbent sheet
- 45ml tap water (for balloon inflation) via white port

Preparation

1. Two nurses are required for insertion of the device. The role of the assistant is to support their colleague and ensure the patient is kept informed throughout the procedure, whilst assisting with balloon inflation. Gather equipment for the procedure
2. Explain the procedure to the patient and obtain patient consent.⁷
3. Perform hand hygiene.
4. Position the patient in a left lateral position, or so that access to the rectum is possible.
5. Don personal protective equipment.
6. Position absorbent sheet under patient. Some fluid seepage is anticipated around the catheter on insertion.
7. Perform a digital rectal exam to check for adequate anal tone, faecal impaction or any in-dwelling device. If impaction is present discuss appropriate treatment with medical officer. Do not insert catheter in this case.
8. Remove gloves and perform hand hygiene.
9. Assemble equipment and open kit.
10. Fill supplied syringe with 45ml water or saline.
11. Attach syringe to the white inflation port of catheter.
12. Unfold the length of the catheter to lay flat on the bed. Ensure tubing is free from obstruction and no traction applied. Connect the collection bag to the connector at the end of the catheter. Write the date/time of insertion onto labels provided and place on space at end of bead strap.

Insertion

1. Perform hand hygiene
2. Don gloves and insert lubricated index finger into the blue finger pocket for catheter guidance during insertion. The finger pocket is located above the position irrigation



line.

3. Liberally coat the balloon end of the catheter with lubricant.
4. Grasp the catheter and gently insert the balloon end through the anal sphincter until the balloon is beyond the external orifice and well inside the rectal vault. The finger can now be removed OR it may remain in place during initial balloon inflation
5. Inflate the balloon with up to 45mL of water or saline by slowly depressing the attached syringe. NEVER INFLATE WITH MORE THAN 45mL OF FLUID.



6. Remove finger (if still inserted). There is a green and red PROTECT dome indicator on the catheter. The green dome indicator will indicate (inflate) once the balloon has reached the optimal level of fluid for the patient's anatomy. If this is less than 30mL of fluid, withdraw the fluid and reposition the balloon in the rectal vault. After repositioning, refill the balloon as above. **Never inflate with more than 45ml of fluid.** The red PROTECT indicator will start to indicate (inflate) if the balloon is overfilled beyond 45mL. If the red balloon is inflated, assess the patient's position, fully deflate the balloon and repeat the inflation process until the green dome indicates optimal fill.
7. Remove the syringe from the inflation port, and gently pull on the soft catheter to ensure the balloon is secure in the rectum, and that it is against the rectal floor. Take note of the position indicator line relative to the patient's anus. It is important to regularly check the location of the position indicator line to determine any movement of the retention balloon in the patient's rectum as this may indicate the need for the device or the balloon to be repositioned.
8. If the device is expelled, deflate the balloon fully, rinse the balloon end of the catheter and perform a rectal exam to ensure no stool is present in the rectum. If no stool is present, reinsert the device following the procedure above. If the device is expelled more than three times, discontinuation of the device should be considered
9. Position the length of the catheter along the patient's leg to avoid kinks/obstructions. Hang the bag by the bead strap on the bedside at a position that is lower than the patient.
10. Dispose of waste appropriately.
11. Remove gloves and perform hand hygiene.
12. Document date of insertion, change by date (no more than 29 days) and removal of catheter in inpatient notes and Nursing Care Plan, along with frequency and volume of irrigation.

3.4.2 Irrigation

Requirements

- 50ml Luer lock syringe
- Absorbent sheet
- 50-100ml Lukewarm tap water

Procedure

1. Perform hand hygiene and don PPE.
2. Position patient on side to avoid catheter compression and obstruction of outflow.

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3. Elevate the patients head (tilt the bed) to promote gravity driven drainage.
4. Place absorbent sheet under the patient to collect possible overflow/spillage.
5. Using a luer lock syringe filled with 50mLs of lukewarm tap water, attach syringe to the blue irrigation/medication port (labelled IRRIG/Rx). **Do not irrigate through the white inflation port.**
6. Slowly depress the plunger.
7. Discard syringe after each use.
8. If repeated flushing with water does not return flow of stool through the catheter, inspect the device to ensure there is no external obstruction (such as kinks, solid faeces). If no obstruction is found, use of the device should be discontinued.
9. Remove gloves and perform hand hygiene
10. Document on the fluid balance chart the volume of flush solution (as input), and volume and consistency of stool drainage (as output). Ensure accurate nursing handover of ongoing plan.

3.4.3 Medication Administration

Requirements

- 50ml Luer lock syringe
- Absorbent sheet
- Required medications as prescribed

Procedure

1. Perform a 3-point identification check of the patient and the 6 rights of medication administration
2. Fill the syringe with 10mL of lukewarm water.
3. Attach the syringe to the blue irrigation/medication port marked IRRIG/Rx and flush the irrigation line with 10mL of water.
4. Prepare a new syringe with prescribed medication.
5. At the black indicator line, position the cinch clamp loosely on the catheter.
6. Connect the syringe with the medication onto the blue irrigation/medication port marked IRRIG/Rx and administer medication. **Do not administer medication through the white inflation port.**
7. Immediately flush the irrigation line with at least 50mL of water.
8. Tighten the cinch clamp on the catheter to ensure no flow through the catheter (ensure the second notch is engaged, squeeze tightly using forefinger and thumb of both hands to ensure a good seal)
9. Allow the medication to remain in the rectum for the time indicated by the prescribing physician.
10. Remove the cinch clamp
11. Attach a new syringe and flush the irrigation line marked IRRIG/Rx with 10mL of water.
12. Dispose of waste equipment appropriately.

3.4.4 Stool Sampling

This is carried out using the Flexi-Seal™ System blue inbuilt sampling port ⁷

Requirements

- Personal Protective Equipment (PPE)

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- Non-Luer lock syringe

Procedure

1. Perform hand hygiene.
2. Don PPE.
3. Open the dark blue sample port cap.
4. Press the tip of a non-luer lock syringe through the slit of the sampling port into the catheter interior
5. Withdraw the stool sample into the syringe and reseal the port.
6. Dispose of waste appropriately.
7. Remove PPE and perform hand hygiene.

3.4.5 Removal

Requirements

- Personal Protective Equipment (PPE).
- Absorbent sheet
- 50ml Luer lock syringe.

Procedure

1. Explain the procedure to the patient.
2. Perform hand hygiene.
3. Don PPE.
4. Position the patient in a left lateral, knee to chest position if possible.
5. Place absorbent sheet beneath patient.
6. The retention cuff must be fully deflated prior to removing the catheter. Attach the syringe to the white inflation port (with the red/green PROTECT domes) and slowly withdraw all the fluid in the retention balloon.
7. Disconnect the syringe and discard.
8. Grasp the catheter as close to the patient as possible and slowly slide out of the anus.
9. Dispose of waste appropriately in yellow bin for medical waste.
10. Remove gloves and perform hand hygiene.
11. Document removal in patient record. Ensure accurate nursing handover of ongoing plan.

Related Documents

Policy:

WA Department of Health (DOH) Consent to treatment policy MP1075/22
 North Metropolitan Health Service (NMHS) Consent to Treatment Policy
 NMHS Patient Identification and procedure matching
 SCGOPHCG Patient Identification
 SCGH Hospital Policy Procedure Matching
 OPH Hospital Policy Procedure Matching.
 NMHS Chaperone Policy (NMHS COC13)

Guideline

NMHS Hospital Falls Management Guideline (HG018)

References

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Printed or personally saved electronic copies of this policy are considered uncontrolled. Refer to the SCGOPHCG Policy Hub for current controlled electronic policies.

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